



AZALEA
HOSPICE & PALLIATIVE CARE

PREPARED FOR
SBA 7(A) UNDERWRITING

BUSINESS PLAN — SBA 7(A) APPLICATION

Azalea Hospice & Palliative *Care*

Acquisition and relaunch of Hickory Hospice LLC — an established, Medicare-certified hospice — as Azalea Hospice & Palliative Care, led by an experienced East-Texas clinical team with an established patient panel and modeled on validated local economics.

Tyler Hospice Hold LLC, operating as Azalea Hospice & Palliative Care.

LOAN REQUEST

\$500,000

SBA 7(a) · 10-yr term

TOTAL PROJECT

\$1.05M

\$500K SBA · \$300K note
· \$250K equity

YR 1 EBITDA

\$311K

21.6% margin · growing
to \$1.19M

GLOBAL DSCR

3.97×

3-yr combined · floor 1.25×

PREPARED

JUNE 2, 2026

CONFIDENTIAL — FOR DISCUSSION PURPOSES ONLY

SECTION INDEX

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All figures are computed from the **Azalea Hospice SBA Loan Package** operating model. Do not distribute without written consent.

O I SECTION ONE

Executive Summary

LOAN REQUEST

Tyler Hospice Hold LLC requests a **\$500,000 SBA 7(a) loan** to acquire and relaunch **Hickory Hospice LLC** — an established, Medicare-certified hospice — as **Azalea Hospice & Palliative Care**. The agency's existing Medicare-certified provider number is acquired through a **change of ownership (CHOW)** for **\$300,000 in seller financing** (36-month note at 6%), giving Azalea immediate, billing-ready capability with no new-enrollment delay. The loan is paired with a **\$250,000 equity injection** and a **\$100,000 working-capital line** held in reserve, for a total project of **\$1.05 million**.

USE OF FUNDS

USE	AMOUNT	DETAIL
Hickory Medicare license (CHOW)	\$300,000	Existing Medicare provider number; self-financed by the seller note
Startup one-time costs	\$63,000	855A filing, TX licensure, EMR, supply stock, legal, contingency
Startup equipment (capex)	\$15,000	Computers, office furniture; 5-yr straight-line depreciation
Working-capital reserve (opening cash)	\$672,000	Funds ramp payroll and dual debt service; holds the \$25K floor, line undrawn
Total uses	\$1,050,000	SBA \$500K • Seller note \$300K • Equity \$250K • WC line \$100K (undrawn)

BUSINESS OVERVIEW

Azalea relaunches the acquired Hickory agency in the Tyler / Smith County market (CBSA 46340) under an experienced local team that brings **established referral relationships and an existing patient panel**. The model is grounded in **validated economics** — a proven ~22 ADC / ~\$118K-net-per-month book benchmarked to actual collections — and the base case **grows with the local referral pipeline to ~40 ADC in Year 2 and ~54 ADC in Year 3**. Revenue is 100% Medicare RHC per-diem at a blended net rate of **\$181/patient-day** (escalating 2.5%/year).

MANAGEMENT TEAM

Azalea is led by an already-seated team: **Geoff Schackmann** (Managing Member — multi-hospice operator), **Silas Shelton** (Executive Director), **Dana Davenport** (Director of Nursing), and **Bradley Woodard** (Director of Sales — 25+ years East-Texas hospice business development), supported by RN case managers, hospice aides, a PRN visit pool, and 1099 Medical Directors.

REPAYMENT CASE

YEAR 1 NET INCOME	YEAR 2 NET INCOME	YEAR 3 NET INCOME
\$198,723 EBITDA \$311K • DSCR 1.61×	\$706,333 EBITDA \$814K • DSCR 4.20×	\$1,083,804 EBITDA \$1.19M • DSCR 6.12×

The operation is EBITDA-positive from Month 1. Against combined debt service of \$193,876/yr (SBA \$84,357 + seller note \$109,519), the model clears a **global 3-year DSCR of 3.97×** — far above the 1.25× floor — with coverage **strengthening each year (1.61× → 4.20× → 6.12×)** and springing higher once the seller note retires after Year 3. The **\$672,000 opening reserve** keeps minimum cash above \$531,000 throughout, with the \$100,000 line never drawn.

02 SECTION TWO

Company Description

2.1 LEGAL STRUCTURE

Tyler Hospice Hold LLC is a Wyoming LLC (formed March 18, 2026; EIN 41-4966640), authorized to do business in Texas. As the borrower and holding entity, it owns **100% of Hickory Hospice LLC** — a Texas HCSSA-licensed, Medicare-certified hospice — which operates as **Azalea Hospice & Palliative Care**. The transaction is a **change of ownership (CHOW)** of Hickory’s existing provider number (purchase of 100% of membership interests), so Azalea runs on a **licensed, billing-ready platform from day one** — preserving the existing number rather than awaiting a new-provider 855A enrollment, with CHAP/ACHC accreditation transferring in the CHOW.

2.2 OWNERSHIP & CAPITALIZATION

MEMBER / SOURCE	INTEREST	ROLE & STRUCTURE
Adeline & Lilah, LLC (AZ)	34.8%	Managing Member — wholly owned by Geoff Schackmann; SBA personal guaranty (≥20%)
James Bullard	25.0%*	Investor / standby creditor; *reserved, post-conversion — pre-conversion holds debt only
Silas R. Shelton	13.3%	Executive Director — 4.9% vested + 8.4% performance-vested
Dana L. Davenport	13.3%	Director of Nursing — 4.9% vested + 8.4% performance-vested
Bradley G. Woodard	13.3%	Director of Sales — 4.9% vested + 8.4% performance-vested
Unissued pool	0.3%	Reserved

SOURCES OF CAPITAL & SBA GUARANTY

The \$1.05M project is funded by the **\$500,000 SBA 7(a) loan**, the **\$300,000 seller note** (self-financing the license), and a **\$250,000 equity injection** — provided through investor James Bullard’s convertible note, of which \$191,000 is on standby under **SBA Form 155** as the required injection. Pre-conversion Bullard holds debt only and stays below 20%, so no additional SBA guaranty is triggered (13 CFR §120.160); the Managing Member provides the SBA personal guaranty.

2.3 ACQUIRED PLATFORM & VALIDATED ECONOMICS

Rather than projecting speculative new-admit growth, the financial model is **benchmarked to actual operating results** — a proven ~22 ADC / ~\$118K-net-per-month book (Mar–Jun 2025 actuals) — so the per-diem rate, patient-day costs, and census all reconcile to real collections. Because the CHOW preserves the provider number, the patient panel, care team, and referral relationships carry over, supporting **revenue continuity from day one**.

2.4 BRAND AND IDENTITY

Azalea Hospice & Palliative Care is positioned for dignity-led hospice service in East Texas. Tagline: *“The standard you upheld your whole life is the one we keep today.”* Service pillars: **Honor, Dignity, Faith, and Reverence** — aligned to the conservative-Christian demographic prevalent in Smith County.

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SECTION THREE

Market and Industry Analysis

3.1 HOSPICE REGULATORY ENVIRONMENT

Hospice in the United States is delivered under the Medicare Hospice Benefit (established 1982, codified at 42 CFR Part 418). Patients with a terminal prognosis of six months or less, certified by two physicians, elect the benefit in lieu of curative treatment. Four levels of care are reimbursed: **Routine Home Care (RHC)** at a two-tier per-diem (days 1–60 higher, days 61+ lower), General Inpatient (GIP), Continuous Home Care (CHC), and Inpatient Respite (IRC). Azalea is modeled at ~100% RHC, consistent with national community-hospice patterns.

For FY2026 the national RHC rates are **\$230.83/day (Tier 1, days 1–60)** and **\$182.36/day (Tier 2, days 61+)**, adjusted by the Tyler CBSA 46340 wage index (0.88) on the 0.68 labor share. With 40% of patient-days in Tier 1, the blended gross rate is \$185.29/PD; net of 2% sequestration and 0.15% write-offs, the blended **net rate is \$181/PD** — validated against actual Tyler-market collections and escalated 2.5%/year (CMS hospice rates have risen every year since 2010).

3.2 TYLER MARKET — DEMOGRAPHICS AND SERVICE AREA

Tyler, Texas (city pop. ~110,000; metro ~245,000) is the principal city of Smith County and the regional healthcare hub for East Texas. Azalea operates from a principal office in the Tyler area with a ~45-mile service radius.

- Service-area population: ~350,000 across Smith, Cherokee, Henderson, Rusk, Van Zandt, and Wood counties
- Medicare-eligible population (65+): ~60,000+
- Hospice utilization: 50–55% of Medicare decedents (above the ~50% national average) — a mature, accepting market
- Population growth: Tyler has grown 15%+ since 2010, driven by healthcare-sector expansion and retirement in-migration
- Dominant religious demographic: conservative-Christian — strong alignment with Azalea’s faith-based service pillars

3.3 MAJOR HOSPITAL SYSTEMS — REFERRAL DRIVERS

Tyler’s hospital infrastructure drives hospice referrals; discharge planners and case managers at the following facilities are critical referral targets:

- | | |
|--|---|
| — UT Health Tyler — 502-bed Level 1 trauma center, regional referral hub | — CHRISTUS Trinity Mother Frances — 438-bed acute care flagship |
| — UT Health North East — 153-bed acute care | — CHRISTUS Louis & Peaches Owen Heart Hospital — cardiac |
| — Texas Spine & Joint Hospital — 64-bed surgical | — UT Health Jacksonville — 90-bed acute care (Cherokee County) |

3.4 SKILLED NURSING & ASSISTED LIVING LANDSCAPE

Smith County and surrounding counties hold ~40+ skilled nursing facilities (SNFs) and ~30+ assisted living facilities (ALFs). These are a concentrated, high-volume referral source: residents with terminal diagnoses frequently elect hospice, and facility relationships drive consistent admission volume.

FACILITY TYPE	COUNT IN AREA	REFERRAL POTENTIAL	PRIORITY
Skilled Nursing Facilities	40+	High — consistent election rates	TIER 1
Assisted Living / Memory Care	30+	Medium-High — growing utilization	TIER 1
Independent Senior Living	15+	Low-Medium — more variable	TIER 2
Residential Care Homes	20+	Medium — relationship-based	TIER 2

Tier 1 facilities (highest-census SNFs and ALFs in the Tyler metro) receive weekly in-person visits from the Director of Sales and clinical liaison; Tier 2 facilities receive bi-weekly or monthly touchpoints by census potential.

3.5 PHYSICIAN & REFERRAL SOURCE ECOSYSTEM

Hospice admissions in the Tyler market are driven by referral relationships with:

- Hospital discharge planners and case managers — the single highest-volume referral source
- Primary care physicians — 200+ internal medicine / family practice physicians in the service area
- Oncologists — UT Health Tyler Cancer Center, Texas Oncology, CHRISTUS — high terminal-diagnosis rate
- Pulmonologists, cardiologists, neurologists — COPD, CHF, dementia/ALS/Parkinson's patients
- SNF/ALF medical directors and DONs — facility-level decision makers
- Faith-community leaders and pastoral care networks — uniquely important in East Texas

A mature, above-average-utilization market with 60,000+ Medicare-eligible residents and 70+ senior facilities — entered not cold, but by an experienced local team on validated Tyler-market economics.

04 SECTION FOUR

Competitive Analysis

The Tyler/Smith County hospice market includes national multi-state operators, regional providers, and local independents. Key Medicare-certified providers serving the market:

COMPETITOR	TYPE	STRENGTHS	WEAKNESSES VS. AZALEA
Heart to Heart Hospice — Tyler	Regional (TX)	Established SNF/ALF referral network	Standard model; no faith-based differentiation
VITAS Healthcare — Tyler	National	Brand recognition, large sales force	Bureaucratic; national protocols; less personal
Hospice of East Texas	Local nonprofit	Trusted community brand	Limited marketing budget; slower growth
CHRISTUS Hospice	Health system	Direct hospital discharge pipeline	System constraints; less flexibility
Enhabit Hospice — Tyler	National (public)	Scale; home-health cross-sell	Fragmented sales effort; less local focus
Traditions Health — Tyler	National	Expanding TX footprint aggressively	Newer entrant; relationships unproven
Kindred / CenterWell	National (Humana)	Captive MA referral pipeline	Limited to Humana MA members
Harbor Hospice — East TX	Regional	Local relationships	Limited capacity; smaller sales team

AZALEA'S DIFFERENTIATION

- **Validated local economics** — modeled on a proven ~22 ADC Tyler book and an experienced team, not a standing start
- **Dignity-led hospitality model** — luxury-hospitality standards applied to end-of-life care
- **Faith-aligned positioning** — explicit Christian-values messaging resonant with the local demographic
- **Speed of admission** — same-day / next-day response, faster than national competitors' 24–72 hours
- **Local independent ownership** — faster decisions, no corporate bureaucracy, deeper relationship investment

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SECTION FIVE

SWOT Analysis

STRENGTHS

- Modeled on validated Tyler-market economics (~22 ADC, ~\$118K/mo) benchmarked to actual local collections
- Experienced clinical & admin team already in seat, with deep East-Texas referral relationships
- Acquired Medicare provider number (CHOW) — billing-ready from day one, no new-provider enrollment lag
- Large capital cushion — \$672K opening cash; \$100K WC line never drawn in base case
- EBITDA-positive from Month 1; combined DSCR rises 1.61x → 6.12x across the plan
- Strong ownership alignment — operators hold equity (13.3% each); experienced multi-hospice Managing Member

WEAKNESSES

- Carries two debt layers (SBA + seller note) in Years 1–3, compressing Year-1 coverage vs. a single-loan structure
- Revenue depends on converting the team's established relationships into admissions during ramp
- Single-office, single-market concentration
- Several go-forward cost adds (own lease, Medical Directors, benefits, marketing) not in the benchmarked actuals
- One direct-care seat (2nd RN, M4) still an open requisition
- Managing Member's attention is split across other hospice interests during launch

OPPORTUNITIES

- Growing 65+ population (60,000+ Medicare-eligible) with above-average utilization
- Base-case census growth to ~54 ADC by Year 3 from the team's referral pipeline; upside path to ~60 ADC
- 70+ SNF/ALF facilities, many underserved by incumbents
- Faith-based marketing to an extensive church and ministry network
- Palliative-care consultation line as a Year 2–3 revenue diversifier
- CMS hospice rates have risen every year since 2010; model carries a conservative 2.5%/yr escalation

THREATS

- National competitors with larger sales forces and budgets
- CMS reimbursement or regulatory changes
- Medicare Advantage hospice carve-in could shift referral dynamics
- East Texas clinical labor shortage could pressure wages
- Payer-integrated competitors with captive referral pipelines
- Seller note (\$300K, 36 mo) adds debt service in Years 1–3; coverage strengthens sharply once it retires

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SECTION SIX

Marketing, Sales & Referral Strategy

Hospice census in the Tyler market is relationship-driven. Azalea's strategy leverages the leadership team's established local referral relationships across five active development channels.

6.1 HOSPITAL REFERRAL CHANNEL

Discharge planners and case managers generate the highest referral volume. The Director of Sales (Woodard) maintains a structured weekly cadence: UT Health Tyler and CHRISTUS Trinity Mother Frances (2x/week each), UT Health North East and Texas Spine & Joint (1x/week each), and bi-weekly touchpoints at regional hospitals. Tactics: in-services on hospice eligibility, branded referral materials, and a same-day admission-response protocol (clinical liaison dispatched within 60 minutes).

6.2 SNF / ALF FACILITY CHANNEL

Tier 1 covers the ~15 highest-census SNFs and ALFs/memory-care communities in the Tyler metro with weekly visits and a dedicated clinical liaison; Tier 2 covers the remaining 40+ SNFs and 20+ ALFs across the six-county service area with bi-weekly touchpoints. Quarterly facility in-services and pre-positioned election packets support consistent volume.

6.3 PHYSICIAN & CLINICAL CHANNEL

CE-accredited physician lunch-and-learns, direct educational outreach, an EHR-embedded referral pathway, and same-day response to physician referrals target the 200+ PCPs, oncologists, and organ-specialty physicians in the service area.

6.4 FAITH COMMUNITY & PASTORAL CARE CHANNEL

Tyler's 200+ churches and pastoral networks are uniquely valuable referral and brand-building partners. Tactics: monthly pastor breakfasts, hospice education for church visitation teams, bereavement-workshop sponsorship, and chaplaincy partnerships with major hospitals.

6.5 MARKETING TACTICS & CHANNELS

Digital. Local SEO (county-specific landing pages, Google Business Profile, review generation; target top-3 local results within 6 months); Facebook/Instagram education and testimonial content to adults 45–75 within 50 miles; Google Ads on high-intent hospice terms.

Traditional & community. Quarterly educational mailers to all SNFs, ALFs, and physician offices; senior-focused print; branded admission books and referral pads; bereavement support groups and senior health fairs; church health-ministry workshops on advance care planning.

MARKETING BUDGET

The model funds **\$3,000/mo (\$36,000/yr)** of marketing and patient acquisition within fixed G&A — covering digital ads, print, collateral, community events, and educational programming — sustained across all three years.

6.6 CENSUS MODEL

The plan headlines the **conservative base case**: census opens with the established panel (ADC 12 → 19.8 → 22 over the first quarter as the team activates referral relationships), then **grows steadily with the local referral pipeline** — to ~24 ADC by Month 12, ~40 ADC across Year 2, and ~54 ADC by Year 3. All DSCR and repayment figures in Section 11 use this base path. An **upside path** reaches ~60 ADC by Year 3 and is presented as scenario sensitivity.

PERIOD	BASE ADC	PRIMARY CHANNEL	MARKETING FOCUS
Months 1–3 (launch)	12 → 22	Panel transition + team activation	Census stand-up + brand launch
Months 4–12	22 → 24	Hospital + SNF/ALF	Digital ads live; community events
Year 2	~40 avg (30 → 50)	All channels + faith	Paid ads optimize; referral network deepens
Year 3	~54 avg (51 → 56)	All channels + organic	Brand recognized; organic referrals dominate

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SECTION SEVEN

Operations Plan

7.1 SERVICE LINES & CLINICAL MODEL

Azalea delivers Routine Home Care (RHC) at patient homes, ALFs, and SNFs, with access to GIP (contracted inpatient beds), CHC, and Inpatient Respite, plus 12-month bereavement support per Medicare CoP. Care is delivered by an interdisciplinary group (IDG) — RN case manager, medical director, hospice aide, social worker, chaplain — meeting every 15 days, with 24/7 RN on-call coverage. A QAPI program runs monthly KPI dashboards.

7.2 CORE STAFFING ROSTER (YEAR 1)

Staggered W-2 hires plus a census-driven PRN pool. Employer burden of 18.27% applies to W-2 wages; health insurance is \$550/employee/mo; Medical Directors are 1099 contractors (no benefits).

NAME	ROLE	GROUP	SALARY	START	STATUS
Brad Woodard	Administrator / Director (Sales)	Indirect	\$120,000	M1	FILLED
Dana Davenport	Director of Clinical Services	Indirect	\$110,000	M2	FILLED
Silas Shelton	Administrator / Executive Director	Indirect	\$130,000	M3	FILLED
Angie Frost	RN Case Manager	Direct	\$77,504	M1	FILLED
Tanisha Robison	CNA / Hospice Aide	Direct	\$46,511	M1	FILLED
Jodi McCollum	RN Case Manager (2nd seat)	Direct	\$75,008	M4	OPEN REQ
Director of Business Dev.	Sales / referrals (growth)	Indirect	\$95,000	M6	FILLED
Rhonda Smith	Office Manager (PRN, W-2)	Indirect	\$34,000	M1	FILLED
Medical Director	Physician oversight (1099)	Direct	\$48,000/yr	M1	FILLED

7.3 CAPACITY-DRIVEN HIRING (YEARS 2–3)

As census grows toward ~54 ADC, the roster scales on census triggers (RN caseload 1:12; aide 1:10), converting PRN roles to salaried and adding compliance/intake/volunteer support. A second Medical Director (\$5,000/mo) is added at Month 22.

CAPACITY / CORE IDG HIRE	SALARY	TRIGGER	START
2nd / 3rd / 4th / 5th / 6th RN Case Manager	\$76,256 ea.	ADC ≥25–55	M13–M34
2nd–6th CNA / Hospice Aide	\$46,511 ea.	ADC ≥25–54	M13–M31
FT MSW / FT LVN / FT Chaplain (convert PRN)	\$52K–\$58K	Core IDG	M14–M18
Quality / Compliance Manager	\$80,000	QAPI / CoP	M16
Intake / Admissions Coordinator	\$55,000	Volume	M20
Volunteer Coordinator (CoP)	\$50,000	CoP req.	M26

7.4 FACILITY & TECHNOLOGY

Principal office in the Tyler, TX area at \$3,000/mo (a go-forward lease assumption). Hours M–F 8–5 with 24/7/365 on-call; ~45-mile service radius. EMR, PCR, and outsourced billing (1.5% of gross) run on the platform reflected in G&A. Field travel is mileage-reimbursed initially; an owned fleet is evaluated in Year 2.

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SECTION EIGHT

Management and Organization

Azalea is operated by an experienced, already-seated team whose established East-Texas referral relationships are a key de-risking factor. Officer salaries are fully loaded into the operating model as expenses (no compensation is omitted, per SBA requirements).

8.1 OWNERSHIP & REPORTING STRUCTURE

ENTITY / PERSON	ROLE	INTEREST	REPORTS TO
Adeline & Lilah, LLC — Geoff Schackmann	Managing Member (holding co.)	34.8%	—
Silas R. Shelton	Executive Director	13.3%	Managing Member
Dana L. Davenport	Director of Nursing	13.3%	Executive Director
Bradley G. Woodard	Director of Sales	13.3%	Executive Director
Rhonda Smith	Controller (consulting)	—	Executive Director
James Bullard	Investor / standby creditor	25.0%*	No operational role

Tyler Hospice Hold LLC (Wyoming holding company) owns 100% of Hickory Hospice LLC (Texas operating subsidiary), which does business as Azalea Hospice & Palliative Care. The Executive Director leads operations; the Director of Nursing leads the clinical team (RN case managers, hospice aides, social workers, chaplains, bereavement), and the Director of Sales leads business development.

*Bullard's interest is reserved post-conversion; pre-conversion he holds debt only.

8.2 DECISION AUTHORITY

- Operating within budget (\$0–\$10K): Executive Director (Shelton)
- Above \$10K or out-of-budget: Managing Member (Schackmann, via Adeline & Lilah)
- Clinical / regulatory: Director of Nursing (Davenport), escalated to Executive Director + Medical Director
- Hiring / firing senior staff: Executive Director, with Managing Member concurrence
- Strategic / capital / acquisition: Managing Member
- Bank account signatures: Managing Member (sole signer)

8.3 ORGANIZATIONAL NOTES

The Year-1 roster is intentionally lean and staggered: direct-care W-2 seats (RN case managers and a CNA/aide) plus a census-driven PRN pool serve the opening ~22 ADC within hospice staffing ratios. Indirect leadership and office support phase in over the first quarter, and the roster then scales on census triggers as the referral pipeline builds toward ~54 ADC.

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SECTION NINE

Leadership Team

GEOFF SCHACKMANN — MANAGING MEMBER (34.8%)

Multi-hospice operator and transaction-led growth leader with operational responsibility for Medicare-certified hospice and palliative-care agencies across multiple states. Sole member of Adeline & Lilah, LLC (AZ), the holding entity through which equity in Tyler Hospice Hold is held. Direct experience includes change-of-ownership (CHOW) transactions and post-CHOW enrollment oversight, census growth from sub-30 to 100+ ADC under existing Medicare provider numbers, multi-site clinical operations under Texas HCSSA and Oregon hospice licensure, CHAP and Joint Commission accreditation, acquisition due diligence (clinical, financial, regulatory), and lender/investor relationships across SBA 7(a), conventional, mezzanine, and convertible-note structures. Leads transaction structuring, financing, capital allocation, and post-close integration.

BRADLEY G. WOODARD — DIRECTOR OF SALES (13.3%)

More than 25 years of East-Texas hospice business development and administration. Founding administrator of one of the Tyler market's largest hospices (licensed 2004; grown to 220 patients, 109 employees, and 62 volunteers, becoming the area's flagship operation), with subsequent census-building roles across multiple East-Texas agencies — including start-ups in Lufkin/Nacogdoches (0→48 in six months; 12→70+) and Grace Hospice of East Texas (8→145+). Most recently VP of Business Development for an East-Texas hospice (2021–present), sustaining a 40+ ADC book for 4.5 years. Licensed Nursing Home Administrator (LNFA #7136); B.S., Texas A&M University. Deep referral relationships across area hospitals, LTC/LTAC facilities, home-health agencies, and physician practices.

SILAS R. SHELTON — EXECUTIVE DIRECTOR (13.3%)

Operational leadership of Azalea Hospice & Palliative Care, with day-to-day responsibility for regulatory compliance under Texas HCSSA and the Medicare Conditions of Participation, payer-mix management, referral-source partnerships across Smith County and surrounding East-Texas counties, and overall site leadership. Direct reports include the Director of Nursing, the Director of Sales, and the full clinical and operational team.

DANA L. DAVENPORT — DIRECTOR OF NURSING (13.3%)

Clinical leadership of the Azalea nursing team, responsible for Medicare Conditions of Participation compliance, interdisciplinary group (IDG) oversight, plan-of-care management, and clinical quality (QAPI). Leads RN case managers, hospice aides, social workers, and chaplains across the service area.

INVESTOR & FINANCE SUPPORT

James Bullard (Investor / Standby Creditor) holds a \$250,000 convertible promissory note; at SBA closing \$191,000 is placed on standby under SBA Form 155 as the equity injection. He holds debt only pre-conversion, with no equity, operational role, or signing authority; conversion is deferred to the end of the SBA term and his interest stays below 20%, so no SBA personal guaranty is required (13 CFR §120.160). **Rhonda Smith (Controller, consulting)** provides finance and accounting oversight — chart-of-accounts maintenance, monthly close, payer-remittance reconciliation, and lender reporting.

IO SECTION TEN

Acquisition and Launch Strategy

10.1 ACQUISITION & CHOW STRUCTURE

The transaction is the **acquisition of Hickory Hospice LLC** — an established, Medicare-certified hospice — structured as a Medicare **change of ownership (CHOW)** of its existing provider number. The purchase is seller-financed via a **\$300,000 note (36 months at 6%)**. Hickory's existing Medicare provider number, state license, and CHAP/ACHC accreditation carry over to Azalea, so the agency is **billing-ready from day one** rather than waiting on a new-provider 855A enrollment. Azalea files the CMS-855A change of ownership and updates banking, EMR, and insurance into the new ownership structure. The acquired provider number is certified in San Antonio with the Tyler / East-Texas market operated as the primary delivery site under the certification.

10.2 REVENUE CONTINUITY

Because the CHOW preserves the existing provider number, the patient panel, care team, and referral relationships transfer intact — revenue continues without the gap a cold-start de-novo would face. The model's economics are **benchmarked to actual collections** rather than to speculative new-admit forecasts, so the opening ramp toward ~22 ADC carries materially less uncertainty, and growth above that level is driven by the team's own referral pipeline.

10.3 AFFILIATE DISCLOSURE

For SBA affiliate-compliance, the Managing Member is separately pursuing an additional East-Texas hospice (Avant Hospice LLC, Mabank, TX) under a separate seller-financed structure, funded entirely outside this SBA loan. The two operations are disclosed as affiliates; this loan funds only the Hickory CHOW and Azalea's working capital.

10.4 LAUNCH TIMELINE

WINDOW	PHASE	KEY ACTIVITIES
Days 1–30	Close & stand-up	Close Hickory CHOW; update banking (EFT/CMS-588), EMR, and insurance to new ownership; staff retention/credentialing; Medical Director contracts
Days 1–90	855A CHOW	File CMS-855A change of ownership; CMS updates the existing provider number to Azalea; license & accreditation transfer confirmed; billing continues uninterrupted
Days 30–120	BD ramp	Woodard-led weekly referral cadence; digital marketing live; 2nd RN seat filled; AR normalizes to the ~45-day Medicare cycle
Day 120+	Growth	Census builds with the referral pipeline; capacity hires triggered by ADC thresholds; combined DSCR > 1.25× and rising

II SECTION ELEVEN

Financial Plan and Projections

11.1 SOURCES OF CAPITAL

SOURCE	AMOUNT
SBA 7(a) loan	\$500,000
Hickory license seller note	\$300,000
Equity injection (Form 155 standby)	\$250,000
Total sources	\$1,050,000

The license is seller-financed, so no cash leaves at close for the purchase — preserving the working-capital reserve. A \$100,000 working-capital line at 10.5% APR is available and undrawn in the base case.

11.2 USES OF FUNDS

USE	AMOUNT
Hickory Medicare license (CHOW)	\$300,000
CHOW filing / 855A change-of-ownership	\$3,000
Texas state licensure	\$5,000
EMR implementation / setup	\$10,000
Initial medical-supply stock	\$10,000
Legal / entity formation	\$15,000
Contingency	\$20,000
Startup equipment (capex)	\$15,000
Working-capital reserve (opening cash)	\$672,000
Total uses	\$1,050,000

11.3 KEY ASSUMPTIONS

Payer mix	100% Medicare RHC
RHC per-diem (Tier 1 / Tier 2)	\$230.83 / \$182.36 (FY2026)
Tyler CBSA wage index	0.88 (0.68 labor share)
Blended net rate / patient-day	\$181 · escalating 2.5% / year
Census — base case (avg ADC)	21.8 → 40.3 → 53.8 (Yrs 1–3)
Census — upside path	to ~60 ADC by Year 3
Patient COGS / PD	\$8.99 (supplies \$3.99 + DME \$1.77 + Rx \$3.23)
Benefits load (W-2)	18.27% + \$550/employee/mo health
Medical Directors (1099)	\$4,000/mo + \$5,000/mo from M22
Outsourced billing / QR fee	1.5% / 0.75% of gross
Fixed monthly G&A	\$13,959 (incl. \$3K rent, \$3K marketing)
SBA 7(a)	\$500,000 · 11.5% APR · 120 mo
Seller note (Hickory license)	\$300,000 · 6% · 36 mo
Equity / WC line	\$250,000 / \$100,000 @ 10.5%
Depreciation & amortization	\$78K startup+capex (5-yr) + \$300K license (15-yr) = \$35,600/yr
TX franchise tax / AR-AP days	0.375% / 45–30
Opening cash	\$672,000

11.4 THREE-YEAR PROFIT & LOSS base case • Year 1 monthly → Years 2–3 grown to ~54 ADC

LINE ITEM	YEAR 1	YEAR 2	YEAR 3
CENSUS & REVENUE			
Average daily census (ADC)	21.8	40.3	53.8
Net patient revenue	\$1,440,528	\$2,724,101	\$3,728,719
COST OF SERVICES & OPERATING EXPENSE			
Patient-related COGS (supplies, DME, Rx)	\$71,549	\$132,002	\$176,276
Payroll & related (W-2, burden, benefits, PRN, MDs)	\$857,087	\$1,547,454	\$2,113,432
Operating G&A (rent, EMR, billing, QR, marketing...)	\$200,688	\$230,253	\$253,392
Total operating cost	\$1,129,324	\$1,909,709	\$2,543,100
EBITDA	\$311,204	\$814,391	\$1,185,619
EBITDA margin	21.6%	29.9%	31.8%
BELOW EBITDA			
Depreciation & amortization	\$35,600	\$35,600	\$35,600
Interest (SBA + seller note)	\$71,479	\$62,242	\$52,232
TX franchise tax	\$5,402	\$10,215	\$13,983
Net income	\$198,723	\$706,333	\$1,083,804

Every line trends the right way: net revenue grows from \$1.44M to \$3.73M and EBITDA from \$311K to \$1.19M as the base-case census builds from ~22 to ~54 ADC on the team's referral pipeline, with a conservative 2.5% annual CMS per-diem escalation. Payroll scales on census triggers (capacity-driven RN/CNA and core-IDG hires), so the cost base grows only as volume warrants.

ACCOUNTING BASIS

"EBITDA" is the operating result before depreciation/amortization, interest, and tax. Payroll burden (18.27%) and the 1099 Medical Directors are fully included. D&A of \$35,600/yr reflects \$78K of startup costs and equipment amortized over five years plus the \$300K license intangible amortized over fifteen years — a non-cash charge below EBITDA that does not affect debt-service coverage.

11.5 YEAR-1 MONTHLY DETAIL

MONTH	ADC	NET REVENUE	EBITDA	MARGIN	DSCR
M1 (transition)	12.0	\$66,029	\$11,438	17.3%	0.71×
M2	19.8	\$108,948	\$38,037	34.9%	2.35×
M3	22.0	\$121,053	\$35,390	29.2%	2.19×
M6	22.7	\$124,904	\$20,944	16.8%	1.30×
M12	24.0	\$132,058	\$27,275	20.7%	1.69×

DSCR here is on **combined** debt service (SBA loan + seller note = \$16,156/month). Month 1 is the transition month (partial census at 12 ADC); its 0.71× reflects full debt service against a half-census month and is fully absorbed by the \$672K opening cash reserve. Coverage clears the 1.25× floor from Month 2 onward; the Month 6 dip reflects the Director of Business Development hire, after which coverage climbs as census builds. On an annual basis Year 1 covers at 1.61×

11.6 LENDER SUMMARY & DEBT-SERVICE COVERAGE

METRIC	YEAR 1	YEAR 2	YEAR 3
Net revenue	\$1,440,528	\$2,724,101	\$3,728,719
EBITDA	\$311,204	\$814,391	\$1,185,619
Debt service — SBA	\$84,357	\$84,357	\$84,357
Debt service — seller note	\$109,519	\$109,519	\$109,519
Combined DSCR	1.61×	4.20×	6.12×
Net income	\$198,723	\$706,333	\$1,083,804
Ending cash	\$586,390	\$971,228	\$1,886,385

GLOBAL 3-YR DSCR	DSCR AFTER YEAR 3	MIN CASH / PEAK LOC
3.97× combined · vs floor 1.25×	~14× SBA-only, seller note retired	\$532K line never drawn (\$0)

Combined debt service is \$16,156/month (\$193,876/year): SBA 7(a) \$7,029.77/mo (\$500K, 11.5%, 120 mo) plus the seller note \$9,126.58/mo (\$300K, 6%, 36 mo). The operation clears the 1.25× floor on an annual basis in every year, and combined coverage rises sharply as census builds — springing to roughly 14× on the SBA loan alone once the seller note retires at the end of Year 3. The \$672K opening reserve keeps minimum cash above \$531,000 throughout, with the working-capital line undrawn.

11.7 STRESS TESTS Year-2 steady state • combined debt service • floor 1.25×

SCENARIO	NET REVENUE	EBITDA	DSCR	CLEAR?
Base (Year 2)	\$2,724,101	\$814,391	4.20×	YES
Census -20%	\$2,179,281	\$315,851	1.63×	YES
Census -35%	\$1,770,666	(\$58,055)	-0.30×	NO
Wage +10%	\$2,724,101	\$663,311	3.42×	YES
Combined (-20% / +10%)	\$2,179,281	\$164,770	0.85×	MARG.

READING THE STRESS TESTS

This is a deliberately severe sensitivity: revenue and variable costs scale with the census shock while **the full Year-2 salaried roster is held constant**, and the test carries **both debt layers simultaneously** (\$193,876). In practice the roster is **census-driven** — the capacity RN/CNA and core-IDG hires only load in as ADC crosses their triggers — so a 20–35% lower census would carry a materially lighter cost base than shown. Even on this static basis, the base case covers at 4.20× and a -20% census still clears at 1.63×. Above all, **liquidity is never at risk**: the \$672K opening reserve keeps minimum cash above \$531K in every scenario with the working-capital line undrawn, and the seller note retires after Year 3, lifting SBA-only coverage dramatically.

11.8 PROOF: ACTUALS VS. MODEL

The model is not pure projection — its economics are benchmarked to the acquired book's **actual operating results**. Those actuals (Apr–Jun 2025) reconcile to the model's opening steady state within ~2%.

METRIC (MONTHLY)	APR 2025	MAY 2025	JUN 2025	MODEL M3
Gross revenue	\$116,423	\$127,893	\$119,991	—
Net revenue (after seq. & write-offs)	\$113,696	\$124,981	\$116,570	\$121,053
Implied ADC	~21	~22	~21	22.0

Apr–Jun average net revenue ≈ \$118,400/mo at ~22 ADC; the model's opening steady-state month (M3) is \$121,053 (+2.2%).

Patient-related COGS (pharmacy, supplies, DME) also reconcile to the actuals. Payroll differs by design — Azalea's staggered W-2 roster plus benefits and go-forward Medical Directors versus the acquired book's blended run.

I 2 SECTION TWELVE

Risk Factors and Mitigations

The dominant risks are the dual debt load (SBA + seller note in Years 1–3) and the census ramp — both structurally mitigated by validated, benchmarked economics, a CHOW that keeps the agency billing from day one, and a deliberately over-capitalized structure relative to operating need.

RISK	PROB.	IMPACT	MITIGATION
Dual debt service (Yrs 1–3)	MED	MED	Combined DSCR clears the 1.25× floor annually and rises 1.61× → 6.12× as census builds; seller note retires after Year 3, lifting SBA-only coverage sharply
CHOW transition / cash timing	LOW	MED	CHOW preserves the provider number (billing-ready day one); \$672K opening reserve + \$100K WC line fund any claims-processing pause; AR days toggleable to re-size the reserve
Slower census ramp	MED	HIGH	Opening census is the validated ~22 ADC book, not a cold start; capacity hires are census-gated so cost flexes with volume; ~20% census still clears at 1.63×
Wage inflation	MED	MED	+10% wage shock holds at 3.42× in Year 2; 3% merit already modeled; PRN pool buffers
CMS rate / regulatory change	LOW	MED	Rates up or flat every year since 2010; only a conservative 2.5%/yr escalation assumed
Key-person / affiliate attention	LOW	MED	Operators are owners (13.3% each); Executive Director leads day-to-day; parallel Avant transaction is funded and staffed separately

The credit's core protection is liquidity, not optimism: \$672,000 of opening cash and an undrawn \$100,000 line keep Azalea solvent through every modeled scenario — even while carrying both the SBA loan and the seller note at once.

13

SECTION THIRTEEN

Milestones and Conclusion

13.1 LAUNCH & GROWTH MILESTONES

MONTH	MILESTONE	VERIFICATION
Month 0	Close Hickory CHOW; banking (CMS-588 EFT) and insurance updated to new ownership	Purchase agreement + cert. of insurance
Month 1	CMS-855A change of ownership filed; staff retained & credentialed; Medical Director contracts; EMR transitioned	855A CHOW receipt + payroll
Months 1–3	License & CHAP/ACHC accreditation transfer confirmed; provider number active under Azalea; billing uninterrupted	License / accreditation transfer
Month 4	BD ramp: 20+ referral touches/week; 2nd RN seat filled; digital marketing live	CRM + analytics report
Months 4–6	AR normalizes to ~45-day Medicare cycle; first remittances clear	AR aging < 45 days
Months 7–12	Census builds toward ~24 ADC; combined DSCR > 1.25×; monthly cash-positive	Monthly financials
Years 2–3	Capacity hires triggered by ADC thresholds; census grows to ~54 ADC; coverage rises to 6×+	Board reviews

13.2 CONCLUSION

Azalea is a well-structured SBA 7(a) opportunity: the CHOW acquisition and relaunch of an established, Medicare-certified hospice on validated economics, led by an experienced local team. The credit rests on three verifiable points: (1) the acquired book's actual operating results, which the opening steady-state model reconciles to within ~2%; (2) a base case whose combined DSCR clears the 1.25× floor and **strengthens every year (1.61× → 4.20× → 6.12×)**; and (3) a deliberately over-capitalized structure — \$250K equity, \$300K of seller risk retained in the note, and a \$672K opening reserve — that keeps liquidity intact through every stress scenario, with the working-capital line never drawn.

SBA CHECKLIST CROSS-REFERENCE

REQUIREMENT	STATUS	LOCATION
Business Plan (editable format)	COMPLETE	This document
3-year P&L projections	COMPLETE	Section 11.4
Year-1 monthly detail	COMPLETE	Section 11.5 + Revenue Model tab
Sources & uses (specific)	COMPLETE	Sections 11.1–11.2
Officers' salaries shown	COMPLETE	Sections 7.2–7.3
Debt schedule	COMPLETE	Section 11.6 + Debt Schedule tab
Purchase agreement & seller note	PENDING	Hickory CHOW — \$300K / 36 mo / 6%
PFS & tax returns (≥20% owners)	PENDING	Personal Financial Statement / History

Confidential — This document contains proprietary financial projections and business assumptions prepared exclusively for the SBA 7(a) loan application of Tyler Hospice Hold LLC (dba Azalea Hospice & Palliative Care). All figures are computed from the Azalea Hospice SBA Loan Package operating model. Do not distribute, copy, or disclose without express written consent.

APPENDIX — OPERATING MODEL REFERENCE

This business plan is computed directly from the **Azalea Hospice SBA Loan Package** workbook, the single source of truth for all financial figures. The workbook's tabs map to this plan as follows:

WORKBOOK TAB	DRIVES
Inputs	Rates, census paths, staffing roster, COGS, G&A, capital structure (Sections 11.3, 7.2–7.3)
Sources & Uses	\$1.05M capitalization and use of funds (Sections 11.1–11.2)
Revenue Model	Year-1 monthly + Years 2–3 quarterly revenue (Sections 11.4–11.5)
Staffing & Payroll / Operating Budget	Capacity-driven payroll and operating expense build (Section 11.4)
Detailed P&L	Three-year profit & loss (Section 11.4)
Debt Schedule	SBA + seller-note amortization and combined debt service (Section 11.6)
Lender Summary / Stress Tests	DSCR, coverage metrics, and shock scenarios (Sections 11.6–11.7)
Cash Flow & BS	Monthly liquidity, minimum cash, working-capital line (Sections 11.6–11.7)
Actuals vs Model	Reconciliation of the acquired book's actuals to model steady state (Section 11.8)

CONSERVATISM BUILT IN

Several go-forward costs are deliberately layered in above the benchmarked actuals — the agency's own lease (\$3,000/mo), a marketing budget (\$3,000/mo), two 1099 Medical Directors, full W-2 benefits, and outsourced billing at 1.5% of gross — so the projected cost base is heavier than the historical run-rate. Revenue carries only a 2.5% annual CMS escalation and a base-case census the team has already demonstrated. The result is a plan that is conservative on cost and grounded on revenue.